

COMMITTEE AMENDMENT

HOUSE OF REPRESENTATIVES

State of Oklahoma

SPEAKER:

CHAIR:

I move to amend HB3644 _____
Of the printed Bill
Page _____ Section _____ Lines _____
Of the Engrossed Bill

By deleting the content of the entire measure, and by inserting in lieu thereof the following language:

AMEND TITLE TO CONFORM TO AMENDMENTS

Amendment submitted by: Preston Stinson

Adopted: _____

Reading Clerk

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

PROPOSED COMMITTEE
SUBSTITUTE
FOR
HOUSE BILL NO. 3644

By: Stinson

PROPOSED COMMITTEE SUBSTITUTE

An Act relating to venous thromboembolisms screening and treatment; creating the Blake Burgess Act; requiring certain hospitals to develop and implement policies and procedures regarding venous thromboembolisms; mandating training for the rendering of appropriate medical attention for persons at risk of forming venous thromboembolisms; requiring the State Department of Health to contract with a private entity to establish a statewide venous thromboembolism registry; providing requirements; requiring hospitals to report certain information regularly to the statewide venous thromboembolism registry; requiring the private entity to provide regular reports to the Department on such data; requiring the Department to provide to the Governor and the Legislature a specified report; providing requirements for report; providing applicability; amending 63 O.S. 2021, Section 1-890.2, which relates to definitions in the Continuum of Care and Assisted Living Act; adding definitions; amending 63 O.S. 2021, Section 1-890.3, as amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025, Section 1-890.3), which relates to promulgation of necessary rules, nursing care component, and adult daycare component; requiring assisted living facilities to provide a consumer information pamphlet containing specified information to residents; providing for

1 noncodification; providing for codification;
2 providing an effective date; and declaring an
3 emergency.
4

5 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

6 SECTION 1. NEW LAW A new section of law not to be
7 codified in the Oklahoma Statutes reads as follows:

8 This act shall be known and may be cited as the "Blake Burgess
9 Act".

10 SECTION 2. NEW LAW A new section of law to be codified
11 in the Oklahoma Statutes as Section 1-630 of Title 63, unless there
12 is created a duplication in numbering, reads as follows:

13 Each hospital with an emergency department and each ambulatory
14 surgical center shall:

15 1. Develop and implement policies and procedures for the
16 rendering of appropriate medical attention for persons at risk of
17 forming venous thromboembolisms which reflect evidence-based best
18 practices relating to, at a minimum:

19 a. assessing patients for risk of venous thromboembolism
20 using a nationally recognized risk assessment tool,
21 and

22 b. treatment options for a patient diagnosed with venous
23 thromboembolism; and
24

1 2. Train all nonphysician personnel at least annually on the
2 policies and procedures developed under this section. For purposes
3 of this section, the term "nonphysician personnel" means all
4 personnel of the licensed facility working in clinical areas and
5 providing patient care, except those persons licensed as health care
6 practitioners. In no case shall this be construed to require
7 personnel to practice outside the scope of his or her certification,
8 license, or qualification.

9 SECTION 3. NEW LAW A new section of law to be codified
10 in the Oklahoma Statutes as Section 1-631 of Title 63, unless there
11 is created a duplication in numbering, reads as follows:

12 A. The State Department of Health shall contract with the
13 state-designated health information exchange to establish and
14 maintain a statewide venous thromboembolism registry to ensure that
15 the performance measures required to be submitted under subsection B
16 of this section are maintained and available for use to improve or
17 modify the venous thromboembolism care system, ensure compliance
18 with nationally recognized guidelines, and monitor venous
19 thromboembolism patient outcomes.

20 B. Beginning July 1, 2027, each hospital with an emergency
21 department shall annually report to the statewide venous
22 thromboembolism registry information containing nationally
23 recognized venous thromboembolism measures and data on the incidence
24

1 and prevalence of venous thromboembolisms. Such data shall include
2 the following information:

3 1. The number of venous thromboembolisms identified and
4 diagnosed;

5 2. The age of the patient;

6 3. The ZIP code of the patient;

7 4. The sex of the patient;

8 5. Whether the patient is a resident of a licensed nursing or
9 assisted living facility;

10 6. Whether the venous thromboembolism was fatal;

11 7. How the diagnosis was made, such as by using imaging
12 modalities; and

13 8. The treatment that was recommended for the venous
14 thromboembolism.

15 C. The Department shall require the state-designated health
16 information exchange to collect data from each hospital with an
17 emergency department on the performance measures required under
18 subsection B of this section. The state-designated health
19 information exchange shall provide to the Department regular reports
20 on the data collected.

21 D. By June 1, 2027, the Department shall submit to the
22 Governor, the President Pro Tempore of the Oklahoma State Senate,
23 and the Speaker of the Oklahoma House of Representatives a detailed
24 report on incidents of venous thromboembolism using inpatient and

1 outpatient data for services provided between July 1, 2026, and June
2 30, 2027. The report shall provide analyses of all of the
3 following:

4 1. Age category, initial primary diagnosis and procedure, and
5 secondary diagnoses, readmission rates for inpatients, admission
6 rates for venous thromboembolism for which the patient had an
7 ambulatory surgery procedure, and emergency department visits for
8 venous thromboembolism linked to any previous admission;

9 2. Whether the venous thromboembolism was present upon
10 admission;

11 3. The incidence of venous thromboembolism procedures reported
12 on the agency's website; and

13 4. The principal payor, the sex of the patient, and the
14 patient's discharge status.

15 E. The state-designated health information exchange operating
16 the registry shall only use or publish information from the registry
17 for the purposes of advancing medical research or medical education
18 in the interest of reducing morbidity or mortality.

19 SECTION 4. AMENDATORY 63 O.S. 2021, Section 1-890.2, is
20 amended to read as follows:

21 Section 1-890.2. As used in the Continuum of Care and Assisted
22 Living Act:

1 1. "Assisted living center" means any home or establishment
2 offering, coordinating or providing services to two or more persons
3 who:

- 4 a. are domiciled therein,
- 5 b. are unrelated to the operator,
- 6 c. by choice or functional impairments, need assistance
7 with personal care or nursing supervision,
- 8 d. may need intermittent or unscheduled nursing care,
- 9 e. may need medication assistance, and
- 10 f. may need assistance with transfer and/or ambulation;

11 2. "Board" means the State Board of Health;

12 3. "Commissioner" means the Commissioner of Health;

13 4. "Continuum of care facility" means a home, establishment or
14 institution providing nursing facility services as defined in
15 Section 1-1902 of this title and one or both of the following:

- 16 a. assisted living center services as defined in the
17 Continuum of Care and Assisted Living Act, and
- 18 b. adult day care center services as defined in Section
19 1-872 of this title; ~~and~~

20 5. "Department" means the State Department of Health;

21 6. "Pulmonary embolism (PE)" means a condition in which part of
22 the clot breaks off and travels to the lungs, possibly causing
23 death; and

1 7. "Venous thromboembolism (VTE)" means deep vein thrombosis
2 (DVT), which is a blood clot located in a deep vein, usually in the
3 leg or arm. The term can be used to refer to DVT, pulmonary
4 embolism, or both.

5 SECTION 5. AMENDATORY 63 O.S. 2021, Section 1-890.3, as
6 amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025,
7 Section 1-890.3), is amended to read as follows:

8 Section 1-890.3. A. The State Commissioner of Health shall
9 promulgate rules necessary to implement the provisions of the
10 Continuum of Care and Assisted Living Act. Such rules shall
11 include, but shall not be limited to:

12 1. A uniform comprehensive resident screening instrument to
13 measure the needs and capabilities of residents in all settings and
14 to determine appropriate placements of residents. Such assessment
15 shall include recognizing the signs and symptoms of venous
16 thromboembolism (VTE) and techniques for providing an emergency
17 response;

18 2. Physical plant requirements meeting construction and life
19 safety codes, with provisions accommodating resident privacy and
20 independence in assisted living centers and in assisted living
21 components of continuum of care facilities based on the variable
22 capabilities of residents;

1 3. Staffing levels responsive to the variable needs of
2 residents, with provisions for sharing of staff between components
3 in a continuum of care facility;

4 4. Minimum standards for resident care including, but not
5 limited to, standards pertaining to medical care and administration
6 of medications. Standards pertaining to medication administration
7 shall, at a minimum, require the assisted living center or continuum
8 of care facility to:

- 9 a. provide or arrange qualified staff to administer
10 medications based on the needs of residents,
- 11 b. follow medication administration orders from a
12 qualified health care provider,
- 13 c. ensure that medications are reviewed monthly by a
14 Registered Nurse or pharmacist and quarterly by a
15 consultant pharmacist,
- 16 d. maintain medication administration records and
17 document all medication administration in such
18 records, and
- 19 e. have medication storage and disposal policies;

20 5. Standards for measuring quality outcomes for residents;

21 6. Provisions for individualized services chosen by and
22 designed for each resident;

23 7. Provisions to prohibit facility staff from disclosing a
24 resident's financial information to third parties without written

1 consent of the resident or the designated representative of the
2 resident;

3 8. Procedures for inspections and investigations of licensed
4 entities to ensure compliance with the Continuum of Care and
5 Assisted Living Act and rules promulgated by the Commissioner;

6 9. Enumeration of resident rights and responsibilities to be
7 observed by each facility and its staff. Such resident rights shall
8 include the freedom of choice regarding any personal attending
9 physicians and all other providers of medical services and supplies,
10 providing that the minimum standards are met by the provider
11 pursuant to the Continuum of Care and Assisted Living Act, without a
12 financial penalty or fee charged by the assisted living center;

13 10. Provisions for a surety bond or deposit from each applicant
14 in an amount sufficient to guarantee that obligations to residents
15 will be performed, with provisions for reduction or waiver of the
16 surety bond or deposit when the assets of the applicant or its
17 contracts with other persons are sufficient to reasonably ensure the
18 performance of its obligations;

19 11. Assisted living facilities shall provide a consumer
20 information pamphlet to residents upon admission. The pamphlet
21 shall contain information about venous thromboembolism (VTE), risk
22 factors, and how residents can recognize the signs and symptoms of
23 VTE. This shall be provided by the National Blood Clot Association
24 at no cost to the facility;

1 12. Provisions for the development of a consumer guide or
2 similar resource to be posted on the Internet website of the State
3 Department of Health to assist individuals and families in
4 understanding the services provided by assisted living centers and
5 to compare and select a facility;

6 ~~12.~~ 13. Provisions for posting results of routine inspections
7 and any complaint investigations of each assisted living center on
8 the Internet website of the Department. Such information shall be
9 regularly updated to include the facility's plan of correction and
10 to indicate when a violation of a licensing regulation was corrected
11 by the facility; and

12 ~~13.~~ 14. Provisions requiring execution of a plan of care and a
13 resident service contract with the resident or resident's
14 representative.

15 B. The nursing care service of a continuum of care facility
16 shall be subject to the requirements, procedures and remedies set
17 out in the Nursing Home Care Act, including provisions relating to
18 resident rights.

19 C. The adult day care component of a continuum of care facility
20 shall be subject to requirements and procedures specified under the
21 Adult Day Care Act.

22 SECTION 6. This act shall become effective July 1, 2026.

23 SECTION 7. It being immediately necessary for the preservation
24 of the public peace, health or safety, an emergency is hereby

1 declared to exist, by reason whereof this act shall take effect and
2 be in full force from and after its passage and approval.

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